جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : عزه ربيع رمضان غانم
المحافظة : القاهره **الحي / المدينة :** المقطم **العنوان :** المقطم
رقم البطاقة : 28407012202049 **رقم التليفون :** 01102148277 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 01-07-1984 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: عزه ربيع رمضان غانم
National ID: 28407012202049

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Protecting Program for Women's Health
البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

2-Patient ID:
1- Name :
2-ID number:
Sex: Female
3- Medical number: 83206290
Old Med :
Patient Age : 41 Y
Date: 1482-2024

BM-Y

Family history		Phone	01120287569	BMI	
Marital status		NRS Score		GROUP	
Co morbidity		Menopausal status		Initial STAGE	
REASON OF MDT					
Date of MDT	29/01/2025				

Clinical data

Breast Imaging :
[illegible] suspicious process

Mets Workup :
Ct cap: free
Bone scan : free

BX	ER	PR	Her 2 Neu	SISH	Ki67
DC		B/S	2	Not Amplified	70

cT 4b	N 3c	M 0
Sonomammography		

PATIENT HISTORY
Post mdt: start NAC (Taxotere x4 ---> Ac x4) guided by clinical response then consider Lt. MRM
T MRM: free surgical margin
N 0/16 DNG

MDT OUTCOME & PLAN

FOR PORT
ADUVANT ZOLADEX FOR 5 YEARS CONSIDER EXTENDED
ZOMETA 6 MONTHS

MEDICAL ONCOLOGY			
SURGERY		RADIOLOGY	
Psychiatric		PATHOLOGY	
		Nutrition	

COMMON SIGNATURE

Patient Name	: الأستاذه / عزه ربيع رمضان غانم	Request No.	: 13009314
Patient No.	: 28407012202049	Request Date	: 28/12/2024
Path No.	: 5232/24	Verification Date	: 15/01/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 40 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, post NACT.
Nature Specimen: Left modified radical mastectomy specimen.

Gross Examination:
* Procedure: Left MRM measured 24x21x6 cm with covering skin ellipse measured 19x18 cm.
* Tumor extent: An ill defined rubbery greyish white tumor bed site.
* Size: 10x9x4 cm.
* Skin involvement: Nipple & areola were unremarkable grossly, the skin showed an elevated area measured 1.5x1 cm.
* Distance from margins: 2 cm away from deep margin, and 0.5 cm from skin.
* Lymph nodes number and size: 16 L.Ns up to 3.5x1.5 cm.
* Designation:
1-10: Tumor bed site.
11: Tumor bed site + elevated area.
12: Skin elevated area.
13: Deep margin.
14: Nipple.
15-18: Lymph nodes.
Dissected by: Dr. Samia Ali.

Microscopic Examination:
* Histological type: No residual invasive tumor.
* Ductal carcinoma in situ: Not detected.
* Skin involvement: Free.
* Lymph node involvement: Free lymph nodes in (0/16) lymph nodes, most of the nodes showed therapy effect in the form of necrotic tissue, fibrosis, xanthogranulomatous inflammation.

Verified by :
Dr.Toka Medhat

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / عزه ربيع رمضان غانم	Request No.	: 13009314
Patient No.	: 28407012202049	Request Date	: 28/12/2024
Path No.	: 5232/24	Verification Date	: 15/01/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 40 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

* Resection Margins: Free deep surgical resection margin.
* Histologic response to therapy: Complete pathological response to therapy.
* Microcalcifications: Positive.

Diagnosis:
Left breast cancer post NACT, MRM specimen:
- No residual invasive tumor.
- Free deep surgical resection margin.
- Free skin.
- Free lymph nodes in (0/16) lymph nodes, most of the nodes showed therapy effect.
- Complete pathological response to therapy.
- Stage: ypT0, ypN0.

Signature
Dr. Walaa Ghonaim
Dr. Yasmine Essam

Verified by :
Dr.Toka Medhat

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
امانة العواكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Azza Rabeea Ramadan Ghanem	Date : 15/05/2024
Exam : C.T CHEST WITH CONTRAST	Age : 40 Year(s)
Medical ID: 1482/2021	Pat. ID: 53829

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. ***Chest:***

- Left breast shows diffuse skin thickening with heterogenous parenchyma
- Left axillary level I ,II and III pathologically enlarged lymph nodes the largest measures 4.9x 2.8 cm.
- Prominent left internal mammary lymph nodes
- Nonspecific pleural based nodule seen at the middle lobe
- No definite pulmonary masses or cavitary lesions
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. ***Abdomen and pelvis:***

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Tiny gall bladder radiodense stone
- Multiple para aortic small lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Right ovarian cyst 3.2x2.6 cm likely functional
- No abdominal or pelvic masses or collections.

3. ***The scanned skeleton:***

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Azza Rabeea Ramadan Ghanem	Date : 15/05/2024
Exam : C.T CHEST WITH CONTRAST	Age : 40 Year(s)
Medical ID: 1482/2021	Pat. ID: 53829

- L2 vertebral body osseous lytic lesion is seen
- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- ***Left breast malignant looking mass lesion with pathological ipsilateral axillary lymph nodes as described.***
- ***L2 osseous lytic lesion for bone scan correlation.***
- ***No CT evidence of definite pulmonary or hepatic deposits.***

Amina O Mohamed

Signature Prof. Menna Tallah Hanafy
20/05/2024 07:59 م

CT Scan
[[DOCTYPE:CT_SCAN]]

PATIENT NAME: AZZA RABEA RAMADAN GHANEM
GENDER: Female
DATE : 27-May-2024 11:29 AM

PATIENT ID: 10124242755
AGE: 39 y
ORDER: 2863109

99MTC-MDP BONE SCAN

Desired Information: -
skeletal survey.

Technique:
Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

No hot spot that account for osseous deposits could be detected allover the surveyed skeleton.
Degenerative changes are seen over dorso-lumbar vertebrae.
Arthritic changes are seen over both knees.
Both kidneys are normally visualized.

Conclusion:

No definite evidence of metastatic osseous deposits.

Reported By,

Prof. Huda Fathy, Nuclear Medicine Consultant **27-May-2024 02:50 PM**

www.Baheya.org 16602

Patient Name	: الأستاذه / عزه ربيع رمضان غانم	Request No.	: 13003451
Patient No.	: 28407012202049	Request Date	: 24/04/2024
Path No.	: 1356/24	Verification Date	: 01/05/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 39 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Ali n ostafa

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass and left axillary lymph node
Nature of Specimen: Tru-cut biopsy

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:
- IPT (Positive control showed appropriate reactivity):(both specimens)
- ER: Strong nuclear reaction in 50% of tumor cells (Allred score 7/8).
- PR: Strong nuclear reaction in 80% tumor cells (Allred score 8/8).
- Her2: Moderate complete membrane reaction in >10% tumor cells (score 2+) for SISH
- Ki67: 70% nuclear reaction of tumor cells.

Conclusion:
1)Left breast cancer, Tru-cut biopsy:
Invasive duct carcinoma, Grade II
2)Left axillary lymph node:Positive for metastatic deposits
IPT:(in both specimens):
- ER: Positive (score 7/8).
- PR: Postive (score 8/8).
- Her2: Equivocal (score 2+) for SISH
- Ki67: Positive in 70% of tumor cells.

Verified by :

غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Women's Health Outreach Program

Name: Azza Rabea Ramadan Ghanem
Date: 24/04/2024

Bilateral Full field Digital Mammography

Clinical summary: 40 years old lady with negative family history complaining of left breast palpable lump of couple of month duration.

Technique

Mediolateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- Heterogeneously dense breast parenchyma (ACR c).
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- Ultrasound:
- No cystic or solid masses seen.
- Normal skin/nipple/areolar complex.
- Non-specific axillary lymph nodes detected.

Left breast

- Heterogeneously dense breast parenchyma (ACR c).
- UOQ distortion with no definite masses or suspicious clusters of microcalcifications.
- Thickened skin and coarsen trabeculations.
- Ultrasound:
- UOQ, 2-3 o'clock two irregular antiparallel hypoechoic solid mass lesions with moderate surrounding and internal vascularity on Color Doppler study seen at:
- Index mass: 10 cm from nipple, 4 mm deep to the skin, measuring 32 x 23 mm.
- Mass b: 6 cm from the nipple, 3 mm deep to the skin, 26 mm from the index, measuring 28 x 16 mm. (corresponding to the mammographic findings)
- Thickened skin reaching 5 mm.
- Normal nipple/areolar complex.
- Multiple pathological axillary lymph nodes are noted showing globular shape and lost hilum, the largest on level 1 measured 27 x 24 mm and on level 2 measured 32 x 20 mm. No nodes detected at level 3 or supra-clavicular.

Impression:

Right breast: (BIRADS 1); normal study.

Left breast: (BIRADS 5) UOQ bifocal suspicious process with ipsilateral pathologically enlarged axillary level 1 & 2 lymph nodes. Tru-cut biopsies were taken at same setting from the index mass as well as one of level 1 lymph nodes.

Recommendation:

Right breast: annual screening

Left breast: histopathological correlation.

Best regards,
Dr. Amal Hegazi, M.Sc.

Mammogram
(DOCTYPE:MAMMOGRAM)

B. HEYA

Radiology Department

PATIENT NAME: AZZA RABEA RAMADAN GHANEM
GENDER: Female
DATE : 05-Jun-2024 11:50 AM

PATIENT ID: 10124242755
AGE: 39 y
ORDER: 2877461,2877822

CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY

INDICATION:
- left breast pathologically proven IDC

TECHNIQUE:
- Contrast Enhanced Spectral Mammography CC and MLO views

Report :

Left breast :
- ACR : C
- Almost diffuse breast pleomorphic microcalcification .
- Left breast UOQ focal assymtery with distortion .
- BY CESM show multiple scattered upper outer and inner pathologically enhancing irregular speculated masses the largest measure 4 x 4 x 3.8 cm
- Corresponding by ultrasound as diffuse parenchymal distortion with multiple ill difened hypoechoic intraductal soft tissue lesions from 10 to 3 o'clock largest lesion seen at 2-3 o'clock measure about 2 x 2.7 cm .
- Multiple pathologically enlarged left axillary , infraclavicular , sub pectoral lymphadenopathy largest measure about 3.7 x 2.5 cm
- Few enlarged supra clavicular LNs the largest measure about 0.4 x 0.4 cm .

Right breast :
- ACR : C
- NO speculated mass lesions or suspioces microcalcification .
- Contrast Enhanced Spectral Mammography showed faint back ground parenchymal enhancement.
- No pathological breast mass or non mass enhancement.
- Normal skin thickness and contour.
- Normal nipple areola complexes.

Mammogram
[[DOCTYPE:MAMMOGRAM]]

- Right axillary lymph node show single LN with diffuse cortical thickening up to 4 mm .

OPINION:

- Left breast pathologically proven IDC show multiple pathologically enhancing malignant looking lesions as described (BIRADS 6)
- Multiple pathologically enlarged left axillary , infraclavicular , sub pectoral lymphadenopathy.
- No right breast pathological mass or non mass enhancement (BIRADS 2)
- Right axillary indeterminate axillary LN .

Reported By,

Dr. Amira Atef ,Msc 05-Jun-2024 12:36 PM

[[DOCTYPE:MAMMOGRAM]]